



# Document Management System



Sultanate of Oman  
Ministry of Health  
The Royal Hospital  
Department of Nursing

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**Title:** Fall Prevention Policy

## Fall Prevention Policy

### 1. Definition

A **fall** is defined as an event that results in a person coming to rest inadvertently on the ground or floor or other lower level, with or without injury.

## **2.Scope**

All staff members are responsible for implementing the intent and directives contained within this policy, and for creating a safe environment of care. Any staff member, physician, or family member may request that a patient be placed on Fall Precautions.

### 3. Goal:

- Decrease incidence of falls
- Decrease severity of falls
- Increase mobility and function
- Improve environmental safety
- Provide comprehensive assessment
- Enhance staff knowledge on Falls Reduction Prevention Strategy
- Improve the patient's confidence

- **Purpose:**

- The fall prevention policy is intended to objectively and systematically identify patients at risk for fall
- To provide a standardized approach to prevent client falls and injury from falls through the use of a standardized Falls Prevention Strategy
  - To protect patients and promote patient safety
  - To effectively identify and intervene with patients who are at risk for falling
  - To educate patients, families, and staff members on measures to prevent falls and promote safety.

#### **4. Policy Statement/ Overview**

identified as a higher level of risk, more in-depth prevention interventions will be implemented. All patients will be assessed for fall risk during the in-patient and ambulatory service.

**4.1 Fall risk will be assessed during any type of visit and will be reassessed:**

- Within 24 hours of admission to acute or long-term care
- At change in patient's status
- Following a fall;
- Once a week.

**4.2 The Falls Risk Reduction Prevention Strategy** is comprised of four main components and involves all employees in preventing and reducing client falls.

**1. The four components are:**

- **Risk Assessment**
- **Implementation** of standard interventions
- **Customization** of interventions for those at highest risk; and
- **Communication** and education.

1. The Falls Reduction Prevention Strategy uses a multidisciplinary approach with roles and responsibilities across the continuum of care.

**Tools are provided to:**

- Identify clients at Risk for falls
- Initiate preventative approaches;
- Provide appropriate strategies and interventions
- Promote a safe environment for all clients
- Provide learning opportunities for clients, their families and employees; and
- Monitor and evaluate outcomes.
- Meets the requirements of the ACI Required Organizational Practices. (ROP)

**1. Fall Risk Assessment Team Approach**

- Identify Risk of fall
- John Hopkins Fall Scale Communicate Risk/ Humpty Dumpty
- Bedside signage
- Fall Risk ID band
- Nursing Documentation : Document Moderate to High risk & Main Interventions identified
- Review John Hopkins Fall Scale Risk Factors
- Implement Standard Interventions
- Conduct High Risk Assessment: Contributing factors
- Implement High Risk nursing Interventions
- Provide patient/family education

**1. The John Hopkins fall risk assessment , Based on 5 main factors:**

- History of falling
- Age
- Elimination, bowel and urine
- Medication
- Patient care equipment
- Mobility

### 4.3 Review Risk Factors/Lower Risk

#### 1. History of falling

- Assessment goal: To prevent recurrence of fall

##### Intervention strategy:

- Develop a strategy to prevent recurrence
- Develop patient-specific fall intervention plan
- Alert staff about the circumstances of the 1st fall
- Conduct rounds every 2 hours to check on Patient

#### 4.3.2 Age

- Assessment goal: To modulate intervention based on age

##### Intervention strategy:

- Develop a individualized fall prevention plan
- Maintaining a safe environment for all patients (i.e. low-bed, side rails up, adequate lighting, allow attendants if necessary)
- Assess the role of current medications which can contribute to the risk of fall

#### 1. Elimination, bowel and urine

- Assessment goal: To reduce urinary urgency

To manage the alteration in bowel habit

##### Intervention strategy:

- Assess for urinary and bowel management needs
- Avoid “rushing to the bathroom” by providing bowel and bladder routine
- Review medication for urinary and bowel implications.
- Establish an individualized toileting routine
- Promote optimal daily intake of food and fluids.
- Assess for toileting transfers and self-care abilities.

#### 1. Medication

- Assessment goal: To determine side effect of medication which might lead to fall

##### Intervention strategy:

- Consult with physician and pharmacy
- Adjust medications accordingly
- Assessing the side effects of medication on patients regularly
- Limit or eliminate any sedatives, anti-depressants, and anti-psychotics if possible.

## MEDICATIONS ASSOCIATED WITH FALLS

This chart highlights some classes of drugs and the possible adverse effects of each that may increase a patient's risk of falling.

| Drug Class                | Adverse Effects                                                                   |
|---------------------------|-----------------------------------------------------------------------------------|
| Diuretics                 | Hypovolemia, orthostatic hypotension, electrolyte imbalance, urinary incontinence |
| Anti-hypertensives        | Hypotension, syncope                                                              |
| Beta-adrenergic blockers  | Hypotension, syncope                                                              |
| Nitrates                  | Hypotension, syncope                                                              |
| Tricyclic Antidepressants | Orthostatic hypotension                                                           |
| Antipsychotics            | Orthostatic hypotension, muscle rigidity, sedation                                |
| Benzodiazepines           | Excessive sedation, confusion, paradoxical agitation, loss of balance             |
| Antihistamines            | Excessive sedation, confusion, paradoxical agitation, loss of balance             |
| Opioids                   | Hypotension, sedation, motor incoordination, agitation                            |
| Hypnotics                 | Excessive sedation, ataxia, poor balance, confusion, paradoxical agitation        |
| Antidiabetic drugs        | Acute hypoglycemia                                                                |
| Alcohol                   | Intoxication, motor incoordination, agitation, sedation, confusion                |

### 4.3.5 Patient care equipment:

Intervention strategy:

- If wheelchair use observes transferring technique
- Provide appropriate aids, and that they are used correctly
- Remind patient of physical limitations due to IV lines, chest tubes and indwelling catheters, etc....
- Maintain safe and clear environment surrounding patients.

4.3.6 Mobility:

- Assessment goal: To assess impairment of gait and balance

Intervention strategy:

- Refer to physiotherapy for exercise
- Walk patient regularly and provide a safe route to the bathroom
- Remind patient to use call bell when in need of assistance, to use the handrail and plan a route
- Inform family about limitation and plan for fall intervention
- If using pole as walking aid, provide walker and assist with ambulation

4.3.7 Cognition:

- Assessment goal: To improve orientation and acceptance of changed abilities

Intervention strategy:

- Observe frequently
- Frequently reorientation and remind
- Do not leave unattended in diagnostic areas
- Involve family for observations and planning care
- Use non-pharmacological approaches and environmental assessment.

1. **Types of fall**

- **Accidental fall:** Fall that occurs unintentionally (example: slip, trip). Patients at risk for these falls cannot be identified prior to a fall and generally do not score at risk for falling on a predictive instrument.
- **Unanticipated Physiological Fall:** Fall that occurs when the physical cause of the fall is not reflected in the patient's assessed risk factors for falls.. These falls are created by conditions that cannot be predicted before their first occurrence (example: seizure, stroke)
- **Anticipated Physiological Fall:** Fall that occurs in patients whose risk factor score indicated the patient is at risk of falling. Controlled sliding down a wall to the ground or utilization of a physiologic structure is considered a fall. These falls are related to existing and previous risk factors.
- **Intentional fall:** Fall that occurs as a result of a patient who voluntarily alters body position to a lower level.

1. **Injury Severity:**

An injurious fall is any fall in which physical injury occurs, regardless of severity.

**A major injury related to a fall is defined as:**

the following: subdural hematoma, concussion, TBI or behavioral changes. \* The major injury definition includes:

- **None**—patient had no injuries (no signs or symptoms) resulting from the fall; if an x-ray, CT scan or other post fall evaluation results in a finding of no injury
- **Minor**—resulted in application of a dressing, ice, cleaning of a wound, limb elevation, topical medication, pain, bruise or abrasion
- **Moderate**—resulted in suturing, application of steri-strips/skin glue, splinting, or muscle/joint strain
- **Major**—resulted in surgery, casting, traction, required consultation for neurological (basilar skull fracture, small subdural hematoma) or internal injury (rib fracture, small liver laceration) or patients with coagulopathy who receive blood products as a result of a fall
- **Death**—the patient died as a result of injuries sustained from the fall (not from physiologic events causing the fall) repeated

#### 1. Fall Risk Factors:

**Intrinsic Risk:** consist of factors as related to the patient such as medication effect, medical or health issues such as:

- Unstable gait or balance
- Urinary frequency
- Poly-pharmacy/high risk meds (benzodiazepines, etc....
- Impaired vision
- Anemia
- Dehydration
- Arthritis, dizziness, low blood pressure and generalized weakness
- Poor mobility & confusion

**Extrinsic Risk:** consist of conditions related to environment such as:

- Wet or slippery floors
- Equipment in the patient's way
- Poor lighting.

#### 4.7 Most vulnerable Population:

- Pediatric patients
- Older patients, especially > 60 years.
- Most occur while patients are walking, particularly when using toilet or commode.

## 4.8 Responsibilities of Staff

In this section, the responsibilities of the following staff are delineated:

**A. Director General**

**B. Director of Quality Improvement and Patient Safety**

**A. Director of Nursing**

**B. Unit Nurses and Ward Nurses**

**C. Admissions Nurses**

- F. Pharmacists
- G. Physical and Occupational Therapists
- H. Biomedical
- I. Nursing Quality
- J. General Service Management Staff
- K. Education Service
- L. Fall Team

## A. Director General and Director of Quality and Patient Safety:

Are responsible for ensuring that falls and fall-related injury prevention is:

1. A high priority at the facility
2. Promoted across the facility through direct care, administrative and logistical staff
3. Adequately funded to provide a safe environment for patients and staff

## B. Director of Nursing:

Is responsible for:

1. Implement fall prevention programs that treat fall risk factors, not count them.
2. Establishing population-based fall and injury prevention programs at patient//unit/and service levels.
3. Deploying evidence-based standards of practice when possible
4. Emphasizing importance of clinical expertise in determining and managing fall and injury risks
5. Assures interdisciplinary involvement in all levels of fall and injury prevention programs.
6. Overseeing the policy, implementation, and evaluation of fall and injury *prevention program within the Hospital*

## C. Unit Nurses and Ward Nurses

Are responsible for:

1. Making fall and fall-related injury prevention a standard of care
2. Implements fall and injury prevention specific to specialty patients' risk factors, for example
  - a. Telemetry Unit: Identification of postural hypotension in cardio-surgical patients
  - b. Medical Unit: Identification of patients with known diagnosis of dementia

4. Oversee post fall huddle processes for patients who sustain falls during their care
5. Involve interdisciplinary staff in fall and injury prevention program and quality improvement
6. Ensuring equipment on the unit is working properly and receiving scheduled maintenance. This is done in collaboration with facility equipment experts (Attachment X; Equipment Safety Checklist)
7. Ensuring that all nursing staff receive education about the falls and injury prevention program at the facility and understand the importance of complying with the interventions

## **D. Admissions Nurses**

The admissions nurses are responsible to:

1. Complete the fall-risk screening and assessment on admission
2. Notify the unit of any patients admitted due to a fall; fall in the ER; or ambulatory care has history of recent falls (in the last 3 months), anticoagulation, fracture risk of history, head injury.
3. Follow any procedure for patients admitted due to a fall or have fall injury history, such as a specific color armband, ensuring the bed assigned is close to the nursing station, ensuring there is a medical alert in the ER, ambulatory or signage at the bedside, etc.

## **E. Registered Nurses (RN's):**

Are responsible for:

1. Ensuring compliance of fall and fall-related injury interventions
2. Completing fall-risk screening on transfers, following a change in status, following a fall and at a regular interval and ensuring procedures for high fall-risk patients are in use
3. Ensuring that rooms with vulnerable patients are assessed and corrected if necessary for slip and trip hazards
4. Assess factors that make patients more or less at risk for falling.
5. Develop an individualized fall and injury prevention plan of care.
6. Collaborate with interdisciplinary team members for implementation and evaluation of the individualized plan of care.
7. Communicate patient's fall and injury history, risk factors, treatment plan during hand off processes.
8. Implement patient education based on health literacy to assure patient engagement as partner in care, along with family / caregiver as appropriate
9. Assure a safe environment for the patient to protect from injury should a fall occur.



11. Participate in quality improvement to evaluate patient safety and quality of care.

## **F. Clinical Directors, Physicians, residents, intern**

Are responsible for:

1. Identifying and implementing medical interventions to reduce fall and fall-related injury risk
2. Taking into consideration the recommendations of pharmacists regarding medications that increase the likelihood of falls
3. Ensuring all patients are screened for risk factors for osteoporosis and tested if necessary
4. Screening patients for fall-risk using the patient's self-report and the Timed up & Go test (Outpatient Areas)
5. Referring patients, who were recently admitted to the hospital due to a fall, to a pharmacist to review the medication and to physical or occupational therapy to conduct a more thorough assessment of fall risk

## **G. Pharmacists**

Pharmacists are responsible for:

1. Reviewing medications and supplements to ensure that the risk of falls is reduced
2. Notifying the physician and clearing medications with the physician if a drug interaction or medication level increases the likelihood of falls
3. Asking outpatients to list their medications and supplements again and verify the medications and supplements with the list provided by the physician and against the patient record

## **H. Physical and Occupational Therapists**

Physical and occupational therapists are responsible to:

1. Conduct patient assessments of rehabilitation needs and falls risks.
2. Evaluate patient mobility and safety in the patient's environment to ensure safe transfers, mobility, and activities of daily living.
3. Develop, implement and evaluate an intervention program for patients to reduce their fall-risk and injury risk.
4. Contribute to interdisciplinary care planning.
5. Participate in clinical education of staff and program evaluation at the unit-service level.
6. Conduct clinical cross-training with nursing staff for mobility and training.
7. Examine and enhance tools and products at the unit for fall and injury prevention (rolling seated walkers, w/c brake extenders, non-skid seating, etc.)

Are responsible for ensuring that:

1. Fall prevention technology, such as bed and chair alarms, wandering sensors are checked regularly and equipped with devices to prevent falls
2. Devices to secure cords off floors are operational
3. Proper lighting, motion-sensor lighting works.
4. Call lights are easily accessible while in bed, chair, bathroom.

## **J. Nursing Quality**

Is responsible for:

1. Collecting data to ensure that fall and fall-related injury prevention strategies are effective
2. Conducting case-by-case reviews for all falls to ensure that medications are reviewed and prevention measures are recommended
3. Follow up with interdisciplinary treatment teams when requested to recommend prevention strategies for a patient
4. Participating in the Quarterly Falls Aggregate Review

## **L. General Service Management Staff**

The facility management staff is responsible for:

1. Ensuring a safe environment of care by conducting environmental assessments (Attachment A.)

## **M. Education Service/ Nursing Department**

Are responsible for:

1. Developing an education program about falls for all staff
2. Developing competencies for nursing staff about the fall's prevention program
3. Develop a patient / caregiver education program for fall and injury prevention; healthy bones; anticoagulation and falls

## **N. Fall Team: 5999 (physician, Nurse, physiotherapist, risk manager, Nursing Quality)**

Are responsible for:

1. Attend and assist any fall calls in hospital premises
2. Assessing patient condition to determine patient needs evaluation and management.

## **5. Procedure**

- All in Patients: patients will be assessed according to the policy.
- Fall risk screening will be completed by registered nurse (RN). The RN is responsible for establishing and updating the individual plan of care related to safety and fall prevention.
- All interventions will be documented in the electronic patients' record.
- Family, if available, will be consulted for individualizing fall prevention interventions.

#### **5.1.1 Surgical Department:**

- All surgical patients are considered to be at risk for fall due to the effects of surgical preparation, medications received, and the surgical procedure performed.
- Fall risk is mitigated through continuous/frequent monitoring and observation of status pre, intra, and post-operatively.

#### **5.1.2 Operation Theatre fall prevention**

- Due to the effects of medications/ anesthesia, consider all patients coming to OT at risk to fall.
- Ensure the side rails are always up and well fixed.
- When transferring to or from operation table, make sure to have enough staff for moving the patient.
- Make sure to apply strap around all patients, unless contraindication.

#### **5.1.3 Maternity Unit: Post-natal wards/ Antenatal Ward/ Gynae/D. S**

- Ensure that the new born baby is kept in the crib always not in the mother's bed.
- Instruct the mothers for proper handling of the neonates

### **5.2 Implementation of Interventions:**

- Implement a plan of care based on the level of risk within 24 hours of admission and ambulatory service and update as necessary.
- Apply a yellow armband
- Implement appropriate standard safety measures
- Escort or offer wheelchair assistance to a patient who is at risk for falls
- Provide adequate lighting
- Bed/Stretcher should be on the lowest position with bed lock.
- While on stretcher all rails should be raised for patient safety
- If high risk, do not place patient on examination table without attendants.
- Toilet patient regularly, especially prior to procedure. Instruct patient to use emergency call bell in restroom and keep path to bathroom clear
- While in recovery area-patient to be visible and not left alone
- All moderate sedation patients must be monitored according to hospital policy
- Review patient medications and correlate to pharmacy list for medications that place a patient at risk for falls. Educate patient if on medications that increased risk for falls.
- Provide patient/family with pamphlet on fall safety.

### **5.3 Upon admission to Inpatient:**

- Complete the Fall Risk Data Collection and fill the fall risk assessment chart.
- If the patient's score is  $\geq 9$ , or at the nurse's discretion, initiate Fall Precautions.
- If possible write *Fall Precautions* on patient head board in the room.
- Apply yellow *Fall Precautions* armband on patient.
- Place *Fall Precautions* label on patients' Kardex/ file.

- Provide proper education and instructions to patient/ family to follow all measures to prevent incident of falling.
- Provide call bell, side rails, lower bed level, and frequent rounding to patients, etc...
- Strategy:
  - Implement High risk - interventions as appropriate
    - HIGH RISK sign posted (not available in all wards)
    - Nurse assist when mobilizing
    - Night light on
    - Commode at bedside if appropriate
    - Fall" arm bracelet /verbal consent obtained
    - bedrails Up
    - Call system within reach
    - Bed at appropriate height
    - Ensure secure, non-slip footwear with no trailing laces
    - Environment clear/signage of wet floor/ items within reach
    - Walking aids/commodes accessible
    - Assess need for routine toileting
    - Plan exiting/equipment/items on patient's side
    - Patient/family education

#### 5.4 Ongoing risk assessment

- Fall Risk assessment Reevaluation of patient at risk should be done every shift, when there is a change in patient condition, and when patient gets transferred in.
- The **fall assessment** chart needs to be updated at least twice per week, when there is a change in patient's condition, and when patient gets transferred in.
- If patient condition/score changes, fall Precautions may be initiated or discontinued.
- All patients who have had a lower extremity nerve block anesthetic will be automatically placed on Fall Precautions until the nerve block is discontinued. The patient will then be re-evaluated to determine his/her fall risk.

#### 5.5 Customization of interventions for those at highest risk of falls-related injury:

##### 5.5.1 Identify those clients who are at greater risk for falls-related injury due to:

- Age or frailty
- Bone disorders
- Coagulation disorders
- Surgery

##### 5.5.2 Implement appropriate interventions as detailed in the Falls Prevention Strategy.

##### 5.5.3 Modify the environment to reduce the risk of fall-related injury.

#### 5.5.2 General Interventions:

- Environmental safety
- Equipment checks

## 1. Risk Assessment:

Assess clients at risk of a fall through the following:

- Presence of approved Step Safe logo identifiers (fall yellow armbands, sign board)
- Client presentation:
  - Advanced age
  - Weakness or frailness
  - Requiring assistance
  - Assistive devices (walker, cane, crutches)
  - Accompanying equipment (IV pumps, oxygen)
  - Inappropriate footwear
  - Clients/family who have self-identified to a health care provider that they require assistance with standing and/or walking.

## 1. Implementation of interventions:

1. To minimize the risk of client falls, employees should:

- Ask all clients prior to starting a procedure:
- Do you need help standing?
  - Do you need help walking?
- Assist clients with standing and walking as required.
- Practice safe handling techniques when clients have to be transferred from stretchers, beds or wheelchairs.
- Identify and report any potential environmental hazards that could result in a fall such as wet floors, poor lighting, furniture at improper height, etc.
- Communicate to other members of the healthcare team that a client has been identified as being at risk for a fall.

### 6.3.2 If a fall/near fall does occur:

- Go immediately to the client, check for obvious injury and provide appropriate care. Ask for assistance as required
- In case if the fall happens in the outpatient department call the physician, assess the client and involve the Emergency Department physician and shift the patient to ED for further management. Ask for assistance as required from the fall team
- Reassess the Fall Scale risk assessment
- Determine whether the client's risk score has changed requiring additional interventions.
- Report the occurrence in the Adverse Event Reporting System.
- Document what occurred in the nurse's progress notes including: patient appearance at time of discovery, patient response to event, evidence of injury, location, medical on call notification, medical/nursing actions. Communicate to all shifts that the patient has fallen and is at risk to fall again.
- Notify the patient's relatives.
- Conduct/ Complete Post Fall Analysis as soon as possible after the fall. This should include the charge nurse, RN involved in the patient's care.
- Report the occurrence in the patient file (Doctor Document in the progress note and nurses in the Kardex).

### 6.3.3 General Interventions:

- Environmental safety

## **7.0 Emergency Department Fall Prevention**

7.1 All Emergency Department patients will be assessed for fall risk by the Triage Nurse.

7.1.1 These criteria will be assessed:

- Fall within 30 days
- Dizziness
- Confusion
- Current Seizures
- Age
- Medications

7.1.2 Fall Precautions are to be initiated in the ED if the patient meets criteria.

## **10. Customization of interventions for those at highest risk of falls-related injury**

10.1 Identify those clients who are at greater risk for falls – related injury due to:

- Age or frailty
- Bone disorders
- Coagulation disorders
- Surgery

10.2 Implement appropriate interventions as detail in the Falls Prevention Strategy.

10.3 Modify the environment to reduce the risk of fall-related injury.

## **11. Communication and Education**

- Communicate consistently and regularly with clients, family members and the healthcare team to reduce falls and injury from a fall.
- Provide clients and their families with falls prevention education.
- Communicate the results of the Falls Risk Assessment to the healthcare team, client, and their family.
- Encourage clients/family to inform their healthcare provider if they are at risk of falling.

## **12. General Interventions:**

- Environmental safety
- Equipment check

## **13. Pediatric Fall Prevention Assessment/ Humpty Dumpty Assessment for the inpatient and Ambulatory Service Fall Prevention**

### **13.1 Definition**

usage is one of the several tools developed to assess fall risk in pediatric patients **aged 0-18 years.**

### **13.2 Description of the scale**

The scale includes 7 parameters with grading criteria based on fall risk (**Appendix 1**). Each parameter has a maximum grading score of 3 to 4. The overall minimum score for the scale is 7 and maximum score is 23. No zeroing can be done on HDFS. Patients with a score of 12 and above are considered at high risk for fall.

## **6.0 HDFS Parameters & Clinical Implication**

### **6.1 Age**

**Assessment Goal:** Identify children at risk and tailor interventions accordingly.

#### **Interventions:**

- Keep patient in a suitable cot, according to the age.
- Provide appropriate clothing to the child
- For new parents, educate them on how to handle the newborn

### **6.2 Gender**

**Assessment Goal:** Male paediatric patients are at higher risk of fall than female

#### **Intervention:**

- Educate parents not to leave their kids unattended in the playroom or the cot
- Ensure patients do not climb chairs
- Ensure the floor is dry all the time
- Keep the call bell within patients/ parents reach.
- Explain to the parents the fall prevention strategies

### **6.3 Diagnosis**

**Assessment Goal:** Explore patients at risk of fall secondary to the clinical condition

#### **Intervention:**

- Maintain proper handover of patients' diagnosis and fall risk score
- Assess patients' condition for any deterioration in clinical condition on daily basis.
- Refer patients to appropriate medical services, when needed..

### **6.4 Cognitive impairment**

**Assessment Goal:** Asses patient awareness of self and environment

#### **Intervention:**

- Assess patient's age and cognitive ability
- Orient patient / parents to the ward, staffs, facilities available and the use of call bell.
- Educate parents not to leave the child unattended
- Place side rails up at all the time.

### **6.5 Environment Factor**

## **Intervention:**

- Ensure the floor is dry
- Keep side rails up
- Keep bed wheels locked
- Remove unnecessary equipments from bedside.
- Protect patient by closing gaps in beds.
- Ensure bed header and footer are in place.
- Maintain good lighting and leave the night light on.
- Assist patient while using assistance devices.
- Educate parents to report wet floor.

### **13.3 Children at high risk of fall**

All Paediatric patients are at risk of fall. However, some exhibit more risk than others. The following list involves patients at risk of fall

- Preschoolers
- Gender : Boys to girls ratio ( 2:1)
- Cognitive impairment from sedation, anesthetist, disorientation, developmental delay.
- Children with CNS disorders.
- Children with sensory impairment like vision impairment
- Children under ten are twice at risk of fall compared to the general population
- Children with disabilities and minimal mobility might be at greater risk
- Children on wheel chair, regardless of the cognitive abilities, are at risk of fall from wheelchair
- Children 19-24 months old
- Diagnosis of respiratory/pulmonary/ETN/common neurological disorders
- Children on medications that causes LOC
- Had been NPO for more than 24 hours
- Related to equipment tripped over furniture, fall out of the crib.
- Children with challenging/impulsive disorders
- Likelihood of frequent toileting
- Post op restrictions like pain/casts/splints/ mobility aids

### **13.4 Types of fall in paediatrics**

- **Anticipated Physiological/ intrinsic:** patient's diagnosis or characteristics may predict their likelihood of fall (i.e. medication, unsteady gait, etc).
- **Unanticipated physiological/intrinsic:** Unpredictable fall when no previous history and risk factors based on the fall assessment are identified (e.g in case of seizure & stroke)
- **External/Accidental:** Unintentional fall due to rolling of patient out of bed, tripping or slipping of the patient
- **Developmental:** This type is common in infants/ toddlers as they are learning to walk, pivot or turn.

### **13.5 Preventive Strategies Based on Humpty Dumpty Score for Inpatient & Outpatient**

#### **Settings**

### **13.6 Low risk standard protocol (Score 7-11)**

- Orientation to room
- Level the bed in low position. Keep brakes on, with the side rails up
- Assess large gaps such as that patient could get extremity or body part entrapped. Use additional safety method e.g sheet.



- Assess elimination needs and assist as needed
- Assess for adequate lighting, leave night light on.
- Environment cleans of hazards.
- Educate patient's families/ parents about fall prevention strategies and document the teaching.

### **13.7 High Risk Standard Protocol (Score 12 and above)**

- Identify patients at risk of fall by applying red color ID band.
- Educate patient's parents about fall prevention strategies.
- Accompany/ assist patient with ambulation
- Protect patient by closing off gaps in bed.
- Evaluate medication administration times/ frequency /dosage
- Remove unused items and equipments, and maintain safe environment.
- Keep door open all times unless isolation is required
- Document "fall prevention teaching".

### **13.8 Frequency of Conducting HDFS Assessment in In-patient Settings**

- On admission
- When receiving patients from other wards/ departments
- After a fall incident
- When changes are noted from "Diagnosis" to "Medication Usage" parameters of HDFS.

### **13.9 Frequency of Conducting HDFS Assessment in Out-patient Settings**

- On each visit, for all patients visiting the Out-patient Department

## **14. Fall Prevention Program Evaluation:**

14.1 The Fall Prevention Team will analyze incident reports, post fall flow sheets and post fall analysis forms to determine patterns/concerns and to determine the need for action plans.

1. A multi-factorial interventions program should include some of the following strategies:

- **General Interventions**
  - Environmental safety
  - Equipment checks
- **Individual-specific interventions**
  - Increased observation for “at risk” patients
  - Help with patients at fall risk
  - Interventions for patients with altered mental status and elimination
  - Medication review
  - Tailored exercise program
  - Post falls analysis
- **Education**
  - Patient/family/career education on falls prevention in the wards
  - Education for health care workers
  - Teach the patient how to fall safely.

1.Show him how to protect his hands and face.

2.If he uses a walker or wheelchair, demonstrate how to cope with and recover from a fall.

3.Instruct him to survey the room for a low, sturdy, supportive piece of furniture (such as a coffee table).

wheelchair.

5. Review with the patient how to call for help if he can't get up.

- **Promoting Safety in the Home**

- Secure all carpets and floor coverings around the edges, and tack down worn spots. Never use lightweight, loose mats or throw rugs on bare floors.
- Make sure potential hazards such as stairs are well lighted. Use white paint on either side of a staircase *to enhance visibility*.
- Install strong banisters along all indoor and outdoor steps.
- Use a bedside lamp or low-wattage night-light in the bedroom so that *you don't have to grope around in the dark when getting out of bed*.
- Fit secure handrails in convenient places in the shower and bathtub and next to the toilet. Place nonskid mats inside and alongside every tub and shower.
- Minimize clutter. Store children's toys, especially those with wheels, when not in use.
- Walk carefully if you have a pet, such as dog or a cat, *to avoid tripping*.
- Secure wires from electrical appliances to walls or moldings.
- Store frequently used clothing and other items in places where you can reach them without standing on a stool or a chair.
- Select well-fitting shoes with nonskid soles, don't wear long robes and wear eyeglasses if needed.
- Sit on the edge of a bed or chair for a few minutes before rising, especially if you take a medication that can make you feel dizzy when you stand up.
- Use a walking stick, cane, or walker whenever you feel unsteady—but always inspect the condition of the assistive device before using it.

- **Environmental Safety**

- All units should provide a safe physical environment as part of a general strategy to reduce the risk of falls:
  - Flooring and lighting
  - Clear obstacles and clutter at bedside and along passageways
  - Provide night lights at bedsides, hallways and toilets
  - Provide grab bars in toilets and on slopes
  - Use non-slip flooring and keep floor dry
  - Highlight the edge of steps and slopes

- **Identification Systems**

Staff should be informed of patients at risk of falls through an identification system which may include:

- Fall Risk / notice board behind beds
- Yellow fall identification band.
- Document on electronic patients' record.

## 15. Communicate Fall Risk: Arm ID Band

Apply Fall Risk Arm Band if:

- Risk score greater than 13
- Clinical Judgment if score less than 13
- (Does this person need assistance when getting up?)
- Inform patient of Fall Risk status and process
- Communicate Risk:

Following the post-fall assessment and any immediate measure to protect the patient:

- An incident report should be completed
- A detailed progress note should be entered into the patient's Kardex /ISBAR, record including the results of the post-fall assessment
- Refer the patient for further evaluation by physician to ensure other serious injuries have not occurred
- Refer to the interdisciplinary treatment team to review fall prevention interventions and modify care-plans as appropriate
- Communicate to all shifts during handover report or patient transfer that the patient has fallen and is at risk for recurrent falls

#### 1. Educate patient and family:

- Actively engage patient/family in Fall Prevention
- Tell patient regarding their risk status
- Teach re: fall prevention strategies, e.g. proper footwear & use of assistive devices
- Assist patient/family in obtaining education pamphlets on fall prevention

#### 1. Auditing and evaluation:

- Audits and evaluations will be completed at least annually using audit tools.

#### 1. Measuring Fall Rates

- "Patient Fall" considered as one of the sensitive nursing KPIs which is expected to reflect on the quality of care provided to the patients.
- Fall and its related injury rates, are very helpful and essential measure to illustrate how safe is the patients' overall environment.
- Fall rate is measured by calculating the number of *falls* and the number of *patient's service days* on a given unit over a given period of time such as, 1 month.

Example:

- **First, calculate number of falls occurred during a month, which can be found in the Adverse Event Reporting System.**
  - *For example*, there were 4 falls during month of August in the Medical Unit.
- **Then obtain the number of patient's service days in the same month, which can be found in Statistical Report at Alshifa System.**
  - *For example*, patient's service days during month of August is 2258
- **Divide the number of falls by the number of patient's service days**
  - *For example*, the month of August,  $4/2258 = 0.0018$
- **Multiply the result into 1,000. So,  $0.0018 \times 1,000 = 1.8$ . Thus, the fall rate is 1.8 falls per 1,000 patient's service days**

No. of Fall Incidences

Incidence rate /1000 days =  $\frac{\text{No. of Fall Incidences}}{\text{Total Patient Service Days}} \times 1000$

# Attachment: 1

## Fall Assessment Tool Humpty Dumpty Scale (Paediatric)

| Parameter                   | Criteria | Score |
|-----------------------------|----------|-------|
| Age                         |          |       |
| Less than 3 years old       |          | 4     |
| 3 to less than 7 years old  |          | 3     |
| 7 to less than 13 years old |          | 2     |
| 13 years old and above      |          | 1     |
| Gender                      |          |       |
| Male                        |          | 2     |
| Female                      |          | 1     |

|                                                                                                            |   |
|------------------------------------------------------------------------------------------------------------|---|
| Neurological Diagnosis                                                                                     | 4 |
| Alterations in Oxygenation (Respiratory Diagnosis, Dehydration, Anemia, Anorexia, Syncope/Dizziness, etc.) | 3 |
| Psych/Behavioral Disorders                                                                                 | 2 |
| Other Diagnosis                                                                                            | 1 |
| Cognitive Impairments                                                                                      |   |
| Not Aware of Limitations                                                                                   | 3 |
| Forget Limitations                                                                                         | 2 |
| Oriented to own Ability                                                                                    | 1 |
| Environmental Factors                                                                                      |   |
| History of Falls or Infant-Toddler Placed in Bed                                                           | 4 |
| Patient uses assistive devices or Infant Toddler in Crib or Furniture/Lighting (Tripled Room)              | 3 |
| Patient Placed in Bed                                                                                      | 2 |
| Outpatient Area                                                                                            | 1 |
| Response to Surgery/Sedation/ Anesthesia                                                                   |   |
| Within 24 hours                                                                                            | 3 |
| Within 48 hours                                                                                            | 2 |
| More than 48 hours/None                                                                                    | 1 |
| Medication Usage                                                                                           |   |

|                                                         |   |
|---------------------------------------------------------|---|
| Sedatives(excluding ICU patients sedated and paralyzed) |   |
| Hypnotics                                               |   |
| Barbiturates                                            | 3 |
| Phenothiazines                                          |   |
| Antidepressants                                         |   |
| Laxatives /Diuretics                                    |   |
| Narcotics                                               |   |
| One of the Meds listed above                            | 2 |
| Other Medications/None                                  | 1 |
| <b>Total</b>                                            |   |
|                                                         |   |

Ambulatory assessment scale:

1. Low Risk    2- Moderate Risk    3-High Risk

**Diagnosis:** \_\_\_\_\_

| No       | Categories             | Score ( ü ) |   |   | Comment |
|----------|------------------------|-------------|---|---|---------|
|          |                        | 1           | 2 | 3 |         |
| <b>1</b> | <b>Age</b>             |             |   |   |         |
|          | 0-13                   |             |   |   |         |
|          | 14-50                  |             |   |   |         |
|          | 50-75 & above          |             |   |   |         |
| <b>2</b> | <b>History of Fall</b> |             |   |   |         |

|          |                                      |  |  |  |  |
|----------|--------------------------------------|--|--|--|--|
|          |                                      |  |  |  |  |
|          | One Fall within 6 months             |  |  |  |  |
|          | More than one fall                   |  |  |  |  |
| <b>3</b> | <b>Gait &amp; Mobility</b>           |  |  |  |  |
|          | Stable                               |  |  |  |  |
|          | Unsteady gait                        |  |  |  |  |
|          | Visual or auditory impairment        |  |  |  |  |
| <b>4</b> | <b>Cognition</b>                     |  |  |  |  |
|          | Alert & aware of environment         |  |  |  |  |
|          | Impulsive                            |  |  |  |  |
|          | Disoriented                          |  |  |  |  |
| <b>5</b> | <b>Medication</b>                    |  |  |  |  |
|          | On 1 *high fall risk drug            |  |  |  |  |
|          | On 2 high fall risk drug             |  |  |  |  |
|          | Sedated procedure with past 24 hours |  |  |  |  |
| <b>6</b> | <b>Patient care equipment</b>        |  |  |  |  |
|          | Nil equipment                        |  |  |  |  |
|          | One or two equipment                 |  |  |  |  |
|          | Three or more                        |  |  |  |  |
| <b>7</b> | <b>Environment</b>                   |  |  |  |  |

|              |                                                                                 |  |  |  |  |
|--------------|---------------------------------------------------------------------------------|--|--|--|--|
|              | clutter free bedsides                                                           |  |  |  |  |
|              | Wheel-chairs locked when necessary                                              |  |  |  |  |
|              | Assistive device is available when required                                     |  |  |  |  |
|              | Handrails in patient bathroom are properly secured                              |  |  |  |  |
|              | Door openings flush within the floor for ease of movement for patient equipment |  |  |  |  |
|              | Floor with sign board                                                           |  |  |  |  |
|              | Lighting                                                                        |  |  |  |  |
| <b>TOTAL</b> |                                                                                 |  |  |  |  |

### Key: Score

0-6 –Low Risk    7-12 – Moderate Risk    13-18 – High Risk

**\*High risk medications – Includes PCA/opiates, anticonvulsants, anti-hypertensives, diuretics, hypnotics, laxatives and psychotropics**

### Attachment: 2

### EQUIPMENT SAFETY CHECK LIST



|                                      |                                              |
|--------------------------------------|----------------------------------------------|
|                                      |                                              |
| <b>Wheelchairs</b>                   |                                              |
| <b>Brakes</b>                        | <b>Secures chair when applied</b>            |
| <b>Arm Rest</b>                      | <b>Detaches easily for transfers</b>         |
| <b>Leg Rest</b>                      | <b>Adjusts easily</b>                        |
| <b>Foot Pedals</b>                   | <b>Fold easily so that patient may stand</b> |
| <b>Wheels</b>                        | <b>Are not bent or warped</b>                |
| <b>Anti-tip devices</b>              | <b>Installed, placed in proper position</b>  |
| <b>Electric Wheelchairs/Scooters</b> |                                              |
| <b>Speed</b>                         | <b>Set at the lowest setting</b>             |
| <b>Horn</b>                          | <b>Works properly</b>                        |
| <b>Electrical</b>                    | <b>Wires are not exposed</b>                 |
| <b>Beds</b>                          |                                              |
| <b>Side Rails</b>                    | <b>Raise and lower easily</b>                |
| <b>Side Rails</b>                    | <b>Secure when up</b>                        |
| <b>Side Rails</b>                    | <b>Used for mobility purposes only</b>       |
| <b>Wheels</b>                        | <b>Roll/turn easily, do not stick</b>        |
| <b>Brakes</b>                        | <b>Secures the bed firmly when applied</b>   |
| <b>Mechanics</b>                     | <b>Height adjusts easily (if applicable)</b> |

|                          |                                                                |
|--------------------------|----------------------------------------------------------------|
|                          |                                                                |
| <b>Over-bed Table</b>    | <b>Wheels firmly locked</b>                                    |
|                          | <b>Positioned on wall-side of bed</b>                          |
| <b>IV Poles/Stands</b>   |                                                                |
| <b>Pole</b>              | <b>Raises/lowers easily</b>                                    |
| <b>Wheels</b>            | <b>Rolls easily and turns freely, do not stick</b>             |
| <b>Stand</b>             | <b>Stable, does not tip easily (should be five point base)</b> |
| <b>Footstools</b>        |                                                                |
| <b>Legs</b>              | <b>Rubber skid protectors on all feet</b>                      |
|                          | <b>Steady — does not rock</b>                                  |
| <b>Top</b>               | <b>Non-skid surface</b>                                        |
| <b>Call Bells/Lights</b> |                                                                |
| <b>Operational</b>       | <b>Outside door light</b>                                      |
|                          | <b>Sounds at nursing station</b>                               |
|                          | <b>Room number appears on the monitor</b>                      |
|                          | <b>Intercom</b>                                                |
|                          | <b>Room panel signals</b>                                      |

|                          |                                                                            |
|--------------------------|----------------------------------------------------------------------------|
| <b>Call Bells/Lights</b> |                                                                            |
| <b>Accessible</b>        | <b>Accessible in bathroom</b>                                              |
|                          | <b>Within reach while patient is in bed</b>                                |
| <b>Walkers/Canes</b>     |                                                                            |
| <b>Secure</b>            | <b>Rubber tips in good condition</b>                                       |
|                          | <b>Unit is stable</b>                                                      |
| <b>Commode</b>           |                                                                            |
| <b>Wheels</b>            | <b>Roll/turn easily, do not stick</b>                                      |
|                          | <b>Are weighted and not “top heavy” when a patient is sitting on it</b>    |
| <b>Brakes</b>            | <b>Secure commode when applied</b>                                         |
| <b>Geri/Broda Chairs</b> |                                                                            |
| <b>Chair</b>             | <b>Located on level surface to minimize risk of tipping</b>                |
| <b>Wheels</b>            | <b>Roll/turn easily, do not stick</b>                                      |
| <b>Breaks</b>            | <b>Applied when chair is stationary</b>                                    |
|                          | <b>Secure chair firmly when applied</b>                                    |
| <b>Footplate</b>         | <b>Removed when chair is placed in a non-tilt or non-reclined position</b> |
| <b>Footplate</b>         | <b>Removed during transfers</b>                                            |

|                    |                                           |
|--------------------|-------------------------------------------|
| <b>Positioning</b> | <b>prevent sliding or falling forward</b> |
| <b>Tray</b>        | <b>Secure</b>                             |
| <b>Flooring</b>    | <b>Dry</b>                                |
|                    | <b>Non-Skid</b>                           |
|                    | <b>Clutter-free</b>                       |
|                    | Free of trip hazards                      |

**Attachment: 3****Environmental Rounds**

Area:

Location:

Date:

Reviewer:

|                                                                                                                                 |  |  |  |  |
|---------------------------------------------------------------------------------------------------------------------------------|--|--|--|--|
|                                                                                                                                 |  |  |  |  |
| <b>Exit signs exist and are visible</b>                                                                                         |  |  |  |  |
| <b>Are hallways and corridors clear of obstacles</b>                                                                            |  |  |  |  |
| <b>Furniture and equipment are sturdy and wheels are locked</b>                                                                 |  |  |  |  |
| <b>Furniture and equipment are suitable for the specific needs of the unit</b>                                                  |  |  |  |  |
| <b>Chairs, wheelchairs are suitable</b>                                                                                         |  |  |  |  |
| <b>Commode/seat lifts are properly installed (not loose)</b>                                                                    |  |  |  |  |
| <b>Door handles are secure</b>                                                                                                  |  |  |  |  |
| <b>Handrails in halls present, accessible and properly secured to wall</b>                                                      |  |  |  |  |
| <b>All lights are working properly and areas are well lit</b>                                                                   |  |  |  |  |
| <b>Floor is clean and dry</b>                                                                                                   |  |  |  |  |
| <b>Floor is clear of personal items</b>                                                                                         |  |  |  |  |
| <b>Flooring is level and free of tripping hazards, such as broken tiles or thresholds that are above the level of the floor</b> |  |  |  |  |
| <b>Call bell/light within reach</b>                                                                                             |  |  |  |  |
| <b>Bed in low position</b>                                                                                                      |  |  |  |  |
| <b>Bedside table within reach</b>                                                                                               |  |  |  |  |
| <b>Water within reach</b>                                                                                                       |  |  |  |  |
| <b>Light within reach</b>                                                                                                       |  |  |  |  |
| <b>Room furniture arranged to allow patient space when walking and grab bars/hand rails are accessible</b>                      |  |  |  |  |

|                                                          |  |  |  |
|----------------------------------------------------------|--|--|--|
|                                                          |  |  |  |
| • Door to bed                                            |  |  |  |
| • Bed to commode                                         |  |  |  |
| • Bed to chair                                           |  |  |  |
| • Chair to commode                                       |  |  |  |
| Does patient have footwear present                       |  |  |  |
| Patients clothing does not drag on the floor             |  |  |  |
| Do slippers have non-slip soles                          |  |  |  |
| Are there grab bars next to the toilet                   |  |  |  |
| Is the toilet seat at a height that allows easy transfer |  |  |  |
| Is there a night light in the bathroom                   |  |  |  |
| Other                                                    |  |  |  |
| Other                                                    |  |  |  |
| Other                                                    |  |  |  |
| Other                                                    |  |  |  |

ANY IMMEDIATE SAFETY ISSUES NOTIFY PROPER SERVICE IMMEDIATELY

**Attachment: 4****Environmental Checklist Follow-up**

| Item | Corrective Action | Date Initiated | Responsible Individual | Anticipated Date of Completion | Actual Date of Completion | Remark |
|------|-------------------|----------------|------------------------|--------------------------------|---------------------------|--------|
|      |                   |                |                        |                                |                           |        |
|      |                   |                |                        |                                |                           |        |
|      |                   |                |                        |                                |                           |        |
|      |                   |                |                        |                                |                           |        |
|      |                   |                |                        |                                |                           |        |
|      |                   |                |                        |                                |                           |        |
|      |                   |                |                        |                                |                           |        |

For additional correction Action:

Forward to:

Date:

Returned Date:

Signature:

## FALL SCALE RISK ASSESSMENT AUDIT TOOL

Auditor Name: \_\_\_\_\_

Date of Audit: \_\_\_\_\_

Audit Location: \_\_\_\_\_

Next Audit Date: \_\_\_\_\_

| CHART | IS a fall Scale Risk Assessment Completed with Total Score Documented? |    | COMMENTS |
|-------|------------------------------------------------------------------------|----|----------|
|       | YES                                                                    | NO |          |
| 1     |                                                                        |    |          |
| 2     |                                                                        |    |          |
| 3     |                                                                        |    |          |
| 4     |                                                                        |    |          |
| 5     |                                                                        |    |          |
| 6     |                                                                        |    |          |
| 7     |                                                                        |    |          |
| 8     |                                                                        |    |          |
| 9     |                                                                        |    |          |
| 10    |                                                                        |    |          |
|       |                                                                        |    |          |
|       |                                                                        |    |          |



**Attachment 6:**

**Post-Fall Management**

**Residents / Patients experience a fall with injury are treated based on hospital emergency response protocols.**

## **Residents/Patients Experiencing a Fall with**

- No loss of consciousness
- No injuries to exceed minor hematomas and lacerations
- Conduct post fall team huddle to determine immediate/root cause of the fall within 15 minutes of the fall
- Conduct multifactorial assessment of the patient post fall, per hospital policy and guidelines, considering the following practices:

**A.No Head Trauma**

1.Determine vital signs to include orthostatic blood pressure (sitting/repeat standing blood pressure 1 and 3 minutes; manual cuff) and pulse.

3. Determine circumstances leading to the fall with corrections.
4. Notify the attending physician (or nurse practitioner per hospital, CLC protocol) on as soon as possible.
5. For the 48 hours following the fall:
  - a. Obtain vital signs per hospital policy (usually every 8 hours or every shift)
  - b. Observe for possible injuries not evident at the time of the fall (limb reflex, joint range of motion, weight bearing, etc.)
  - c. Mental status changes

#### **B. Minor Head Trauma**

1. Use the same protocol outlined above.
2. Perform neuro-checks according to policy.
3. Alert the attending physician for any changes.

#### **Additional Measures:**

- Complete post fall note in the patient medical record file
- Complete incident report
- Review fall prevention interventions and modify plan of care as indicated
- Communicate to all shifts that patient has fallen, circumstances and results of the fall event, and changes to the plan of care

**Attachment: 7**

**Post fall Follow-Up Form**

**Date & Time of fall:** \_\_\_\_\_

**Location of fall:** \_\_\_\_\_

**1. Physician notified:** \_\_\_\_\_ **Date & Time:**

**2. Family notified:** \_\_\_\_\_ **Date & Time:**

| <b>Documentation:</b>                                                   | <b>Date/ Time</b> | <b>Initial</b> |
|-------------------------------------------------------------------------|-------------------|----------------|
| Fall documented in progress notes                                       |                   |                |
| Post Fall Investigation Summary Documented in progress notes            |                   |                |
| Fall Risk Assessment Tool Completed                                     |                   |                |
| Individualized Fall Prevention Care Plan reviewed and revised as needed |                   |                |
| Occurrence Report Completed                                             |                   |                |

| <b>Debriefing:</b>                        | <b>Date/Time</b> | <b>Initial</b> |
|-------------------------------------------|------------------|----------------|
| Completed within 72 hours of fall         |                  |                |
| Incident, cause, and Action Plan reviewed |                  |                |
| Action Plan Items Initiated               |                  |                |

**Completed By:**

**Name (Print):**

**Signature:**

**Unit Nurses/Ward Nurse informed:**

**Date:**

### References

| <b>Title of book/ journal/ articles/ Website</b> | <b>Author</b> | <b>Year of publication</b> | <b>Page</b> |
|--------------------------------------------------|---------------|----------------------------|-------------|
|                                                  |               |                            |             |

|                                                                                                                                                              |                                                                                          |      |        |
|--------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------|------|--------|
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| Fall Prevention Strategy.                                                                                                                                    | Labrador-Grenfell Health                                                                 | 2013 | 5 - 33 |
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|                 |                              |      |  |
|-----------------|------------------------------|------|--|
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|-----------------|------------------------------|------|--|

**Written By:** National Oncology Center Nurses Team  
**Checked By:** All Section Head & Unit Nurses, Ms Shakila Quality Nursing  
**Authorized by:** NASRA SALIM SAID AL HASHMY

#### Review Details

|   | Review BY                            | Review Date |
|---|--------------------------------------|-------------|
| 1 | National Oncology Center Nurses Team | 30/10/2019  |
| 2 | Ms Maya Al Ismaili                   | 26/09/2016  |

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